

Medications
nebivolol Oral
olmesartan-amlodipin-hcthiazid Oral

Medical History
Essential hypertension

Musculoskeletal History
None

Musculoskeletal Family History
None

Musculoskeletal Surgery
None

Surgical History
None

Social History
Smoking status - Unspecified

Chief Complaints:
1. Right Shoulder Pain

HPI: This is a 33 year old male who:
1. is being seen for a chief complaint of shoulder pain, involving the right shoulder. This occurred in the context of patient states he was working out and noticed pain on 6/24/25. .

He is presenting with right shoulder pain that he localizes to the superior aspect of the shoulder that began during a workout on June 24. He notes that he began having some throbbing in the area when he was doing presses at the gym. Gradually got worse after that. Has had some improvement but it has not resolved. He has been trying to adjust his activity and use heat, ice, and Biofreeze. He is right-hand dominant and reports no history of any problems with the right shoulder but he did have similar symptoms on the left shoulder in the past.

Vitals:										
Date	Taken By	B.P.	Pulse	Resp.	O2 Sat.	Temp.	Ht.	Wt.	BMI	BSA
07/23/25	Ortega, Renata						67.0 in	273.2 lbs	42.8	2.3
	FIO2									

* Patient Reported

Exam:

Comprehensive, Upper Extremity Neurovascular

Appearance: well nourished

Orientation: Alert and oriented to person, place, time.

Mood: mood and affect well-adjusted, pleasant and cooperative,

Right UE Pulses: normal radial pulse and good capillary refill

Left UE Pulses: normal radial pulse and good capillary refill

RUE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

LUE Peripheral Sensation: intact to light touch throughout peripheral nerve distributions

Coordination: Coordination normal.

Shoulder

Right Shoulder Active ROM:

Forward Flexion: 180 degrees.

Extension: 50 degrees.

Abduction: 180 degrees.

Adduction: 40 degrees.

External Rotation: 45 degrees.

External Rotation in Abduction: 120 degrees.

Internal Rotation: T4 - T8.

Internal Rotation in Abduction: 60.

Left Shoulder Active ROM:

Forward Flexion: 180 degrees.

Extension: 50 degrees.

Abduction: 180 degrees.

Adduction: 40 degrees.

External Rotation: 45 degrees.

External Rotation in Abduction: 120 degrees.

Internal Rotation: T4 - T8.

Internal Rotation in Abduction: 60 degrees.

Right Shoulder Passive ROM:

Forward Flexion: 180 degrees.

Extension: 50 degrees.

Abduction: 180 degrees.

Adduction: 40 degrees.

External Rotation: 45 degrees.

External Rotation in Abduction: 120 degrees.

Internal Rotation: T4 - T8.

Internal Rotation in Abduction: 60.

Skin:

Right Shoulder: skin intact, no rashes or lesions.

Inspection:

Right Shoulder: **AC joint TTP, bicipital groove not TTP, clavicle not TTP, proximal humerus not TTP, subacromial space not TTP, and lateral subacromial space not TTP**

Strength:

Right Shoulder Forward Flexion: Strength: 5/5, normal muscle tone.

Right Shoulder External Rotation: Strength: 5/5, normal muscle tone.

Right Shoulder Internal Rotation: Strength: 5/5, normal muscle tone.

Stability:

Right Shoulder: Stable

Special:

Right Shoulder: **AC cross chest: painful, Empty Can Test: negative, External Rotation Weakness: negative, and Hawkin's impingement: negative**

Left Shoulder Passive ROM:

Forward Flexion: 180 degrees.

Extension: 50 degrees.

Abduction: 180 degrees.

Adduction: 40 degrees.

External Rotation: 45 degrees.

External Rotation in Abduction: 120 degrees.

Internal Rotation: T4 - T8.

Internal Rotation in Abduction: 60 degrees.

Skin:

Left Shoulder: skin intact, no rashes or lesions.

Inspection:

Left Shoulder: Normal alignment, no deformity, no tenderness, no warmth, no masses, no muscle atrophy, no scapular winging, no clavicle deformity, no crepitus

Strength:

Left Shoulder Forward Flexion: Strength: 5/5, normal muscle tone.

Left Shoulder External Rotation: Strength: 5/5, normal muscle tone.

Left Shoulder Internal Rotation: Strength: 5/5, normal muscle tone.

Tests

X-Ray Interpretation Shoulder

Diagnosis: Shoulder Pain, Right - M25.511, M19.011

X-Ray Data:

Date: 07/23/2025

The following film(s) were done in our facility: right axillary view, shoulder AP view, and scapular Y view

X-rays of the right shoulder were ordered and obtained, demonstrating the following findings: There is no fracture. Visual inspection of the shoulder joint reveals no glenohumeral joint space narrowing and decreased AC joint space. Soft tissue exam shows normal soft tissue.

Impression/Plan:

1. **Shoulder Pain, Right**

Pain in right shoulder (M25.511)

Associated diagnosis: Acromioclavicular Arthritis

Plan: Prescription.

meloxicam 15 mg tablet PO

Sig: Take 1 po qd x 2 weeks then PRN- take with food - don't take with ibuprofen or naproxen

Quantity: 30 Tablet Refills: 2

Follow up in 3 weeks

Note:

Symptoms today are most consistent with AC arthrosis. I reviewed the x-ray images with him which show some subtle narrowing at the AC joint. On exam today he has tenderness in the area and pain with cross chest hyperabduction. I recommended that we take a conservative approach to treatment with a 2-week course of nonsteroidals. I have given him a prescription for meloxicam and discussed appropriate use of the medication. I will plan to see him again in 3 weeks to check his progress. I might consider either an injection or an MRI at that point depending on his response.

Staff:

Marla Whitcomb, NP (Primary Provider) (Bill Under)

Electronically Signed By: Marla Whitcomb, NP, 07/23/2025 02:37 PM CDT

Medications

meloxicam 15 mg Oral - tablet
nebivolol Oral
olmesartan-amlodipin-hcthiazid Oral

Medical History

Essential hypertension

Musculoskeletal History

None

Musculoskeletal Family History

None

Musculoskeletal Surgery

None

Surgical History

None

Social History

Smoking status - Unspecified

Chief Complaints:

1. F/U Shoulder Pain, Right evaluated on July 23, 2025

HPI: This is a 33 year old male who:

1. is following up for Shoulder Pain. He was seen on July 23, 2025, at which time X-Ray Interpretation Shoulder was performed and he was prescribed Meloxicam 15 mg tablet (Take 1 po qd x 2 weeks then PRN- take with food - don't take with ibuprofen or naproxen).

The patient presents for further evaluation and management.

He returns today reporting that he is at least 50% better than he was when I saw him originally. He continues to have some lingering discomfort that he localizes to the AC joint. He says he has modified his workout some and is still taking the meloxicam.

Exam:

Shoulder

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Right Shoulder Passive ROM:

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Stability:

Right Shoulder: Stable

Special:

Right Shoulder: **AC cross chest: painful, Empty Can**

Test: negative, External Rotation Weakness: negative, and Hawkin's impingement: negative

Strength:

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Impression/Plan:**1. Shoulder Pain, Right**

Pain in right shoulder (M25.511)

Associated diagnosis: Acromioclavicular Arthritis

Plan: Counseling - Shoulder Pain.

Please refer to the education handout for detailed counseling.

Plan: Prescription Medication Management.

Modify Regimen: Modify prescription medication therapy.

Follow up in 4 weeks**Note:**

I told him to consider completing the month of meloxicam. He has 1 week left. And then he will come off of the medication. I told him it is okay to gradually increase his workouts to see how he tolerates it. I will see him again in 1 month to check his progress. If his symptoms persist we might consider getting an MRI or trying an injection at that point.

Staff:

Marla Whitcomb, NP (Primary Provider) (Bill Under)

Electronically Signed By: Marla Whitcomb, NP, 08/13/2025 08:15 AM CDT